28 October 2026 | Evaluation plan v4 | Discussion draft — no expenditure approval
MEMORANDUM

Panda Vision: evaluating investment in neonatal video support

Action required

Proceed with an existing-evidence, model-based evaluation of the Atherton-led pilot and conditional Queensland expansion. Use the completed analyses to identify when investment is worthwhile; do not present modelled resource savings as observed cash savings. No prospective cohort, randomised trial or structured expert elicitation is a prerequisite.

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Presented by	CMO / neonatal, retrieval, virtual-care and finance partners

Executive summary

1. Decision and comparator. Compare video-supported with telephone-supported specialist consultation: consultation occurs in both arms. The established RSQ/ANTS-NQ escalation pathway is retained. Assess continuation, equipment replacement and rollout from organisational, whole-health-system and societal perspectives. The pilot comprises Atherton, Mareeba and Innisfail; forecast activity is 10–15 consultations per site annually. The 4–5 Atherton episodes describe feasibility, not a causal effect or incidence rate. [ELC2026; USER2026]

2. Existing evidence and assumptions. Use the ELC material, published comparative studies and economic models, public birth/network data, retrieval-cost reports and available routine information. Keep the $10,000 installation estimate marked for invoice/component verification. The 2022 denominators are Atherton 157, Mareeba 134 and Innisfail 195 births. A historical 65-site public roster supports a primary 33-site expansion scenario; it is not a verified 2026 service census. Assumed casemix, geography and uptake remain uncertain. [QH2022; ANZNN2023]

3. Analytical design. Retain the small severity-stratified annual-cohort model. Include failed connections, local care, transfer and later rescue; critical cases receive no transfer-avoidance credit. Shared clinical-effect uncertainty remains correlated across sites. Separate resource costs, NWAU × NEP, QWAU × QEP and actual budgets. NEP26 is $7,418; unverified QEP/local adjustments remain blank. The indexed aeromedical anchor is not a neonatal tariff or guaranteed cash release. [NEP26; QHFUND26; AIR2019; M1]

4. Analyses completed. Reproduced 58 primary deterministic scenarios and eight 20,000-draw probabilistic analyses; five-year budget tables; 65 site projections; nine hypothetical funding comparisons; and 108 exploratory survival/morbidity combinations. Added six equipment-life checks, 18 break-even thresholds and Monte Carlo precision/repeated-seed checks using existing parameters. Plots, complete CSV tables and the results workbook accompany this plan. No clinical parameter has been added. [M1]

5. Current decision signal. The pilot reference PSA has 46.0% probability of resource cost saving, mean +$2,428/year and 95% interval −$41,148 to +$65,642. The illustrative state network gives 50.7%, mean +$135,429 and interval −$1.012m to +$1.809m. A three-year equipment life reduces deterministic pilot net saving to about $80/year. These results show uncertainty, not futility or proof of savings. Unproven mortality gains are not credited in the primary analysis. [M1]

6. Remaining work and release decision. Replace assumptions only where invoices, published updates or ordinary records are available; otherwise retain explicit ranges and structural alternatives. Prioritise clinical-effect transportability, avoidable retrieval resources and equipment life. Complete the connected, locked-environment assurance gates, resolve actual diagnostics and obtain clinical/economic review before publication or investment recommendations. Further prospective evaluation is optional and should address decision-changing uncertainty. [M1]

Engagement and consultation	
Consumers	Family travel, unpaid time and separation are consequences; no consumer-derived utilities or clinical benefits are claimed.
Clinicians and staff	Use available clinical and operational feedback; no requirement for a formal expert-estimation exercise.
External stakeholders	Existing public evidence informs the analysis. No external stakeholder endorsement or service-change approval is implied.

Implications	
Budget	No expenditure approval sought. Separate sunk pilot capital from replacement and rollout. Pilot continuation cash cost is estimated at $7,123/year; full annualised economic cost is $14,052.
FTE	No new establishment approved. Shared specialist support, readiness and training remain explicitly costed assumptions.
Risk	Observational effect evidence, historical births and inferred transport mix limit transferability. Null/harm scenarios and shared uncertainty remain explicit.
Legal	Aggregate existing-evidence modelling only. Obtain appropriate governance/data permissions before individual-record linkage or release of restricted material.
ICT	Tablet/PTZ replacement is a concept, not an approved configuration. Retain biomedical, cleaning, QH Portal far-end control, privacy and cyber checks.
Impact on First People's Health	Access and family displacement are assessed separately. Include survival with major morbidity and its costs; do not use cost saving as the sole criterion.

Strategic Plan alignment

Our care: timely specialist support. Our people: rural readiness. First Peoples health: equitable access. Our sustainability: transparent costs, uncertainty and proportionate engineering.

Attachment/s

1. Analysis supplement v4, results workbook, plots and full tables. 2. CHHHS presentation v4. 3. Repository/Git bundle, Conductor tracks and reusable lean-CI skill. 4. Actual verification receipt and outstanding-gate register. Source identifiers refer to the supplement/register; M1 denotes reproducible model outputs, not an external study.